First Take

Pharvaris N.V. (PHVS)

September 8, 2026

Price: $35.25; Market Cap (M): $2,463; *9/4/2026 Close*

Rating: Buy; Price Target: $60.00

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CHAPTER-3 Delivers 83% Attack-Rate Reduction; Oral Prophylaxis Clears the Injectable Bar; Reiterate Buy

Deucrictibant XR meets primary endpoint in Phase 3 trial in prevention of HAE attacks. This morning, Pharvaris announced that the Phase 3 trial CHAPTER-3 evaluating once-daily deucrictibant extended-release (XR) oral tablet for the prevention of hereditary angioedema (HAE) attacks met its primary endpoint with an 83% reduction in mean monthly attack rate vs placebo (95% CI: 72–90%; p<0.0001) across all three HAE types, and with an 87% reduction in the 80 participants with Type 1 or Type 2 disease in an additional analysis not adjusted for multiplicity (link to deck). All secondary efficacy endpoints were also met with statistical significance. The result clears the 80–85% bar that we had set ahead of the readout and, in our view, secures the oral prophylaxis segment while preserving the "injectable-like efficacy" claim. With the immediate-release (IR) on-demand New Drug Application (NDA) accepted and an April 23, 2027, PDUFA (FDA action) date, and an XR submission planned for 1H27, we believe Pharvaris is positioned to own the only oral franchise spanning both treatment settings. We reiterate our Buy rating and $60 price target on PHVS.

Impressive efficacy "quite similar to the subcutaneous administered medications". CHAPTER-3 (n=85; ≥12 years old) randomized 2:1 to deucrictibant XR 40 mg once daily (n=55) or placebo (n=30) for 24 weeks and is the first prophylaxis Phase 3 to enroll all three HAE types. The estimated attack rate was 0.35 per month on XR vs 2.6 on placebo, off balanced baselines of 3.76 and 3.47 attacks per four weeks, with protection achieved within the first week and sustained through week 24. The depth of response matters more: nearly half of XR patients were attack-free across all 24 weeks, all but one achieved at least a 50% reduction, almost all at least 70%, and roughly two-thirds at least 90%, on pre-specified secondary endpoints. Against company-presented benchmarks of 44% for the only approved oral prophylaxis and 71–87% for injectables, the Type 1/Type 2 result establishes parity with parenteral efficacy in a pill — the comparison that matters commercially. Importantly, in the topline results conference call with management, KOL Dr. Marc Riedl described efficacy that "looks quite similar to the subcutaneous administered medications." Furthermore, retention corroborates the profile as 83 of 85 participants completed and 80 rolled into CHAPTER-4, the long-term open-label extension study of deucrictibant XR 40 mg/day.

Safety was broadly clean; hepatic monitoring is the one label watch item. TEAEs were more frequent on drug (76.4% vs 56.7%) and study drug-related events were 20.0% vs 10.0%, but severity was concentrated: three Grade 3 events and one Grade 4, no treatment-related SAEs, and one discontinuation per arm. Two of the Grade 3 events were asymptomatic liver enzyme elevations slightly above five times the upper limit of normal, both confounded (long-term androgen use, fatty liver). Management expects at most a liver-monitoring section in the label, manageable in our view. In the five normal-C1-INH participants — only 3 on XR — responses were more variable (one attack-free, one roughly 90%, one below 50%), a label-breadth question rather than an efficacy one, in our view. Management framed the ongoing Phase 3 trial CREAATE (Part 1 topline data expected in 1Q27) as non-gating: filing timing is set by the totality of the safety database including ongoing CHAPTER-4, with acquired-angioedema data folded in if ready.

Valuation and Risks. We value Pharvaris using a SOTP and DCF-based methodology based on net income per asset and FCFF, respectively. We assign POS for deucrictibant of 95% in HAE on-demand, 90% in HAE prophylaxis, and 50% in AAE-C1INH. We employ a discount rate of 12% and a terminal multiple of 6.0x. Based on these considerations, we arrive at an implied equity value of approximately $4.1 billion or $60/share. Risks to price target achievement: (1) clinical development risk; (2) competitor risk in a highly competitive market; (3) commercialization

risk; (4) reimbursement risk; (5) pricing risk; (6) capital market and dilution risk; and (7) geopolitical risk tied to ongoing conflicts.

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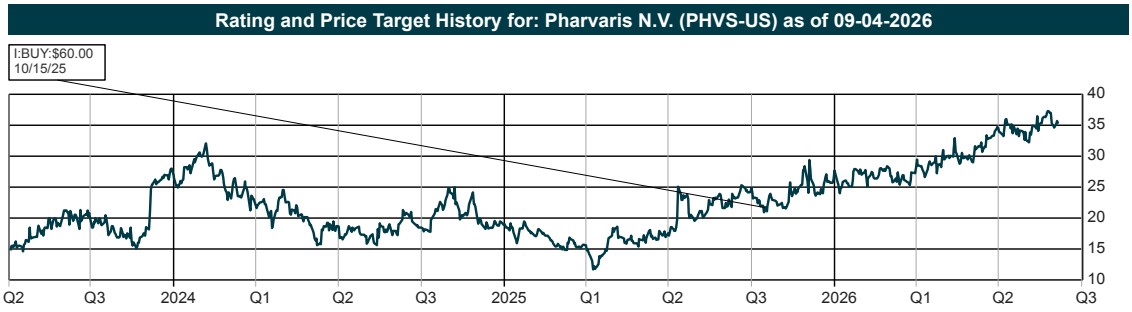
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			IB Service/Past 12 Months	
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Sell	2	0.31%	0	0.00%
Under Review	50	7.78%	20	40.00%

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